TBL – Adrenal Pathology

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Block: Endocrine/Urology/Reproduction
Thread: Pathology

Conflicts of Interest: Dr. Macnow has no conflicts of interest to disclose.

Textbooks

Robbins and Cotran: Pathologic Basis of Disease, 8th ed.—Chapter 24: The Endocrine System

Rubin's Pathology: Clinicopathological Foundations of Medicine, 8th ed.—Chapter 27: The Endocrine System

Learning Objectives

By exam time, you should be able to do the following:

1. Correlate the gross and histopathological features of a pheochromocytoma with its tissue of origin and its clinical manifestations.
2. Describe and identify genetic syndromes commonly associated with pheochromocytomas.
3. Correlate the gross and histopathological features of an aldosterone-producing adrenal cortical adenoma with the clinical findings of Conn syndrome.
4. Correlate the gross and histopathological features of adrenal atrophy (for example, due to autoimmune adrenalitis) with the clinical findings of Addison disease.
5. Describe additional causes of acute adrenal insufficiency, including Waterhouse–Friderichsen syndrome, hypopituitarism, and steroid withdrawal.
6. Correlate the gross and histopathological features of secondary adrenal hyperplasia with the clinical findings of Cushing syndrome or disease.

Biolucida Slide Box

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Slides Used in This TBL Session

Case #1

Adrenal VS 1-1, Gross
Adrenal VS 1-2, H&E

Case #2

Adrenal VS 2-1, Gross
Adrenal VS 2-2, H&E
Adrenal VS 2-3, Gross

Case #3

Adrenal VS 3-1, Gross
Adrenal VS 3-2, PAS

Case #4

Adrenal VS 4-1, Gross
Adrenal VS 4-2, H&E